



OCTOBER 2026 EDITION

Medical Billing Benchmarks 2026: The Four-Silo Guide

Where independent practices lose revenue, the published benchmark for each area, and a monthly self-audit you can run with your own numbers.

INSIDE THIS GUIDE

1. The 2026 benchmark table: MGMA, HFMA, AMA, CMS, CAQH
2. Silo 1: Insurance verification and eligibility
3. Silo 2: Prior authorization fast-track
4. Silo 3: Medical coding accuracy
5. Silo 4: A/R aging and denial recovery
6. Monthly self-audit scorecard and FAQ

Where practices lose revenue in 2026

Most independent practices lose collectible revenue in four places: insurance verification, prior authorization, coding, and accounts receivable follow-up. Each one has a published benchmark, and each one can be measured with data the practice already has.

The pressure is rising. In Experian Health's 2025 State of Claims survey, 41% of providers reported denial rates of 10% or higher, up from 38% in 2024 and 30% in 2022. MGMA puts the typical first-submission denial rate for medical practices at 7% to 8%, and says well-run practices get under 5%.

This guide sets out the benchmark for each of the four areas, the workflow behind it, and the questions practice owners ask most often. Every figure is an industry benchmark with its source named. Use it as a monthly self-audit: pull your own numbers, compare them to the table below, and work on the gap that costs the most.

Who this is for: owners, administrators, and billing managers at independent U.S. physician practices, especially specialties with heavy authorization and documentation requirements such as behavioral health, cardiology, pain management, physical therapy, and podiatry.

The 2026 benchmark table

These are the reference points used throughout the guide. All are published industry benchmarks, not results from any single billing company.

Metric	Industry benchmark	Source
First-submission denial rate	7%–8% typical; under 5% for well-run practices	MGMA, via WeBill Health summary
Providers reporting denial rates of 10% or more	41% (2025), up from 30% in 2022	Experian Health State of Claims 2025
Share of denials from registration and eligibility	24%, the largest single root cause	Optum 2024 Revenue Cycle Denials Index, via Pabau
Clean claim rate	95% or higher	Aegis Health, citing MGMA/HFMA
Days in A/R	30–40 days (HFMA target); over 50 signals a structural problem	HealthCell, DataRovers
A/R over 90 days	Under 10% of total A/R (HFMA); MGMA average about 13.5%	Staffingly, ProMD
Net collection rate	95% or higher	Aegis Health, citing MGMA DataDive
Cost to rework one denied claim	\$25–\$118 in staff time	DataRovers, citing MGMA

Metric	Industry benchmark	Source
Denied claims never reworked	50%–65%	DataRovers, citing MGMA
Prior authorizations per physician per week	About 40, taking 13 hours of physician and staff time	AMA 2025 Prior Authorization Physician Survey

Published denial rates vary widely because publishers count different things: initial versus final denials, claims versus dollars, and different payer markets. Compare your practice against a benchmark measured the same way you measure.

Silo 1: Insurance verification and eligibility

Eligibility errors are the largest single cause of denials, and many of them cannot be recovered. Optum's 2024 Revenue Cycle Denials Index, built from about 124 million claim remits, attributes 24% of denials to registration and eligibility. The same analysis found only 21% of those denials were straightforward to recover and 28% were not recoverable at all.

The 72-hour pre-visit verification window

A 72-hour window gives staff time to fix a coverage problem before the patient arrives. Checking at the front desk leaves no room to correct a terminated plan, a wrong member ID, or a missing referral.

1. **72 hours before the visit:** run an electronic eligibility check for every scheduled patient. Confirm active coverage, plan type, network status, and whether the service needs a referral or prior authorization.
2. **48 hours before:** work the exceptions. Call the patient about inactive coverage, request a referral from the primary care office, or start the authorization.
3. **24 hours before:** send the patient their estimated out-of-pocket amount.
4. **At check-in:** re-run eligibility for any patient whose 271 response returned errors, and scan a new insurance card if one is presented.

EDI 270/271: how electronic eligibility works

The 270 is the HIPAA-standard eligibility inquiry a practice sends to a payer. The 271 is the payer's response, listing coverage status, plan dates, copays, coinsurance, deductibles met and remaining, and benefit limits by service type code. Service type code 30 returns general plan coverage; specialty codes return benefits for a specific service line, which matters for behavioral health and therapy visits.

Automation saves money at scale. CAQH's 2024 Index estimated the medical industry could save about \$11.7 billion a year by moving eligibility checks from phone, fax, and portals to fully electronic transactions. CAQH CORE updated its eligibility operating rule in 2025 to expand the procedure codes and service type codes that support specific benefit inquiries.

A 271 response is only as good as the data in the 270. A transposed date of birth or a missing suffix returns a "subscriber not found" error, so the practice should match names exactly as they appear on the card.

Calculating the patient's share before the visit

The estimate follows the order in which most plans apply cost sharing:

1. Start with the payer's allowed amount for the service, not the practice's charge.
2. Apply any remaining deductible first.
3. Apply coinsurance to the balance after the deductible, or the flat copay if the plan uses one for that visit type.
4. Cap the total at the patient's remaining out-of-pocket maximum.

Worked example (illustrative figures): allowed amount \$250; deductible remaining \$100; coinsurance 20%. The patient owes \$100 toward the deductible plus 20% of the remaining \$150 (\$30), for a total of \$130. If the out-of-pocket maximum has \$90 left, the patient owes \$90.

Collecting that amount at check-in is easier than billing it later, because patient balances become harder to collect the longer they age.

Silo 2: Prior authorization fast-track

Prior authorization takes about 13 hours of physician and staff time per physician each week, according to the AMA's 2025 survey of 1,000 physicians. Physicians reported completing about 40 requests a week, and 32% said their requests are often or always denied. Two in five practices employ staff who work only on authorizations.

What changed in 2026

Since January 1, 2026, the CMS Interoperability and Prior Authorization Final Rule (CMS-0057-F) requires impacted payers to decide expedited requests within 72 hours and standard requests within seven calendar days. Every denial must include a specific reason. The standard window was previously up to 14 days.

The rule has limits a practice should know:

- It applies to Medicare Advantage, Medicaid and CHIP (fee-for-service and managed care), and qualified health plans on the federal exchanges. Commercial and self-funded employer plans are outside it.
- It does not cover prior authorization for drugs.
- Payers must have a prior authorization API live by January 1, 2027. Until then, many requests still go through portals and fax.
- A faster decision is not the same as an approval. Clinical criteria remain with the payer.

Where multi-specialty practices get stuck

Bottlenecks tend to cluster in the same places: imaging and procedures ordered by one provider and performed by another, therapy visit limits that need re-authorization mid-episode, behavioral health services with per-session approvals, and authorizations that expire before the scheduled date. Each one needs an owner and a tracking date.

A zero-delay workflow

1. **At scheduling:** check the payer's authorization list for the CPT code and plan. Flag the appointment if an authorization is required.
2. **Same day:** gather the documentation the payer's criteria ask for, including notes, prior treatments tried, and imaging.
3. **Submit electronically** where the payer supports it, and mark the request as expedited only when the clinical situation qualifies.
4. **Track against the deadline:** 72 hours for expedited and seven days for standard requests to impacted payers. Escalate when a payer misses the window.
5. **Record the authorization number, approved units, and expiration date** in the practice management system, so the claim carries it and staff can see when a re-authorization is due.
6. **On denial:** read the specific reason the payer must now provide, request a peer-to-peer review where available, and appeal with the missing documentation.

The point of the workflow is that no patient is scheduled into a service the payer has not approved, and no approved authorization lapses unused.

Silo 3: Medical coding accuracy

Coding errors are a leading denial driver because every claim must connect a diagnosis to a procedure in a way the payer accepts. In Experian Health's 2025 survey, 50% of providers named missing or inaccurate claim data as the top factor in denials. Coding is where much of that data originates.

The three code sets

Code set	What it describes	Maintained by	Update cycle
ICD-10-CM	Diagnoses: why the patient was seen	CMS and CDC	Annually on October 1
CPT	Procedures and services the provider performed	American Medical Association	Annually on January 1
HCPCS Level II	Supplies, drugs, equipment, and services not in CPT	CMS	Quarterly

Updates in effect now and coming in January

The FY 2027 ICD-10-CM code set took effect October 1, 2026, with 190 new codes, 30 deleted codes, and 4 revised codes. The date of service, not the submission date, decides which code set applies. Notable changes include new site-specific secondary malignancy codes (C78.31, C78.32, C79.83), dedicated plantar fasciitis codes under M67.A, and a split of dilated cardiomyopathy I42.0 into more specific codes, which affects cardiology claims.

The CPT 2027 code set, released by the AMA in September 2026, takes effect January 1, 2027. It includes 299 new codes, 74 revisions, and 80 deletions. The largest structural change replaces the global maternity care codes with separately reported phases. Adaptive behavior (ABA) codes and AI-related services also change.

What certification means for accuracy

Certified coders hold credentials from one of two national bodies. AAPC issues the CPC (professional coding) and specialty credentials; AHIMA issues the CCS and CCS-P. Both require passing an exam and completing continuing education each year, which keeps coders current through the October and January updates. When evaluating a billing partner, ask which credentials its coders hold and whether they specialize in your service lines.

Precision checks that prevent denials

- Code to the highest specificity the documentation supports. Unspecified codes draw medical necessity denials.
- Confirm the diagnosis supports medical necessity for the procedure under the payer's coverage policy.
- Apply modifiers correctly. Modifier 25 (separate E/M on the same day) and 59 or the X modifiers (distinct procedural service) are frequent audit targets.
- Run claims against NCCI edits, which CMS updates quarterly, before submission.
- Audit a sample of each provider's claims every quarter and share the results with the provider.

Silo 4: A/R aging and denial recovery

A claim's odds of being paid fall as it ages, and many denied claims are never worked at all. MGMA estimates 50% to 65% of denied claims are never reworked or appealed, and reworking one claim costs \$25 to \$118 in staff time.

Working each aging bucket

Bucket	What it usually holds	Action
0–30 days	Claims in normal payer processing	Check claim status at day 14–21; fix front-end rejections the same day
31–60 days	Unanswered claims, first denials, underpayments	Call or check portal status; correct and resubmit; compare payment to contract rates
61–90 days	Denials awaiting appeal, missing documentation	File appeals with supporting records; track each payer's appeal deadline
90+ days	Second-level appeals, timely-filing risk, patient balances	Escalate to a senior biller; document every contact; decide on write-off or collections by policy

The HFMA benchmark keeps A/R over 90 days under 10% of total A/R. Watch timely-filing limits in the older buckets: Medicare allows 12 months from the date of service to file a claim and 120 days to request a first-level redetermination, while commercial plans set their own limits, often much shorter.

Root-cause denial tracing

Appealing denials one at a time recovers some revenue but does not stop the next one. Root-cause tracing groups denials so the practice can fix the process that produced them.

1. **Pull 90 days of denials** from electronic remittance advice (835 files).
2. **Group by CARC and RARC code, payer, CPT code, and provider.** Common codes include CO-16 (missing information), CO-197 (authorization absent), CO-27 (coverage terminated), CO-50 (not medically necessary), and CO-29 (timely filing expired).
3. **Map each group to the silo that caused it.** CO-27 points to eligibility, CO-197 to prior authorization, CO-50 and modifier denials to coding, CO-29 to follow-up.
4. **Rank by dollars and recoverability,** and appeal the high-value, recoverable claims first.
5. **Fix the upstream step** with a checklist change, a scheduling rule, or coder education, and re-measure the same code group the following month.

This is what ties the four silos together. Most A/R problems at 90 days started as a missed eligibility check, an expired authorization, or a coding mismatch on day one.

The monthly self-audit scorecard

Pull these seven numbers on the first business day of each month and compare them to the benchmark. Fill in the last column yourself; any row that misses its benchmark points to the silo to work on.

Metric	How to calculate	Benchmark	Silo	Your number
Denial rate	Denied claims ÷ claims submitted	Under 5%–8%	All	
Eligibility denials	CO-27, CO-22, and similar denials ÷ all denials	Trending down	1	
Authorization denials	CO-197 and similar denials ÷ all denials	Trending down	2	
Clean claim rate	Claims accepted on first submission ÷ claims submitted	95%+	3	
Days in A/R	Total A/R ÷ average daily charges	30–40 days	4	
A/R over 90 days	A/R aged 90+ days ÷ total A/R	Under 10%	4	
Net collection rate	Payments ÷ (charges – contractual adjustments)	95%+	4	

Track the same numbers the same way each month. A single month can mislead; three months of movement in one direction is a trend.

Frequently asked questions

What is a good claim denial rate for a medical practice?

MGMA puts the typical first-submission denial rate at 7% to 8%. Well-run practices get under 5%. If your rate is above 10%, you are in the 41% of providers Experian Health found at that level in 2025, and the cause is usually upstream data, not payer behavior.

What is the most common reason medical claims are denied?

Registration and eligibility errors are the largest single cause, at 24% of denials in Optum's 2024 index. Missing or inaccurate claim data, authorization problems, and coding errors follow.

How far in advance should insurance eligibility be verified?

Verify coverage at least 72 hours before the visit. That leaves two business days to fix a terminated plan, a missing referral, or a required authorization. Re-check at check-in for any patient whose first response returned an error.

What is an EDI 270/271 transaction?

A 270 is the standard electronic eligibility request a provider sends to a payer. A 271 is the payer's response, listing coverage status, copays, coinsurance, deductibles, and benefit limits. Together they replace phone and portal checks.

How long does prior authorization take in 2026?

For Medicare Advantage, Medicaid, CHIP, and federal exchange plans, payers must decide expedited requests within 72 hours and standard requests within seven calendar days under CMS-0057-F. Commercial plans and drug authorizations follow their own timelines.

How do I calculate a patient's copay or out-of-pocket cost?

Start with the payer's allowed amount. Apply any remaining deductible, then coinsurance or the copay, and cap the total at the patient's remaining out-of-pocket maximum. The 271 response provides each of these figures.

What is the difference between ICD-10-CM, CPT, and HCPCS codes?

ICD-10-CM codes describe diagnoses. CPT codes describe procedures and services. HCPCS Level II codes describe supplies, drugs, and equipment not covered by CPT. ICD-10-CM updates each October 1, CPT each January 1, and HCPCS quarterly.

What is the difference between AAPC and AHIMA certified coders?

Both are national credentialing bodies. AAPC issues the CPC and specialty credentials, focused on physician and outpatient coding. AHIMA issues the CCS and CCS-P. Both require an exam and annual continuing education.

What is a good days in A/R benchmark?

HFMA targets 30 to 40 days. Above 50 days usually points to a structural denial or follow-up problem.

What percentage of A/R should be over 90 days?

HFMA recommends keeping A/R over 90 days under 10% of total A/R. MGMA's average is about 13.5%.

What is root-cause denial analysis?

It groups denials by reason code, payer, procedure, and provider to find the process step that caused them. Fixing that step prevents the next denial, instead of appealing each one after the fact.

Is it worth appealing a denied claim?

Usually, yes. MGMA estimates 50% to 65% of denials are never reworked, so revenue is written off that could have been recovered. Prioritize appeals by dollar value and by how often that denial type is overturned.

About Svizzera Healthcare Solutions

Svizzera Healthcare Solutions is a medical billing and revenue cycle management company based in Tallahassee, Florida. Its team of 70+ staff, including certified medical coders, brings more than 20 years of U.S. medical billing experience to independent practices in behavioral health, cardiology, pain management, physical therapy, podiatry, and wellness. Each practice works with a named account manager, with no long-term contract.

Free A/R review: send us your current aging report and we will show you where your practice sits against the benchmarks in this guide, bucket by bucket. Request one at gosvizzera.com.

Contact: GoSvizzera.com • 813-713-0957 • info@gosvizzera.com • Tallahassee, Florida

This guide is updated monthly. Follow Svizzera Healthcare Solutions on LinkedIn for each new edition.

Sources

Figures are published industry benchmarks as of October 2026. Several are reported through secondary summaries of the original MGMA, HFMA, Optum, CAQH, and Experian Health data.

- [AMA: 2025 Prior Authorization Physician Survey press release](#)
- [AMA: CPT 2027 code set release](#)
- [Fierce Healthcare: AMA survey on prior authorization denials](#)
- [Healthcare Finance News: CMS Interoperability and Prior Authorization Final Rule](#)
- [Centro CDX: CMS-0057-F scope and limits](#)
- [Firely: CMS-0057-F API timeline](#)
- [Society of Interventional Radiology: CMS releases 2027 ICD-10-CM codes](#)
- [Revelrex: FY 2027 ICD-10-CM changes](#)
- [CAQH CORE: Eligibility & Benefits rule update, July 2025](#)
- [AJMC: 2024 CAQH Index findings](#)
- [Pabau: Optum 2024 Revenue Cycle Denials Index summary](#)
- [Medical Healthcare Solutions: Experian Health State of Claims 2025](#)
- [WeBill Health: 2026 claim denial benchmarks \(MGMA\)](#)
- [Aegis Health: MGMA and HFMA benchmarks](#)
- [HealthCell: Days in A/R benchmarks](#)

- DataRovers: A/R benchmarks
- DataRovers: denial management, MGMA rework figures
- Staffingly: A/R aging benchmarks
- ProMD: MGMA billing benchmarks